# PBM RFI and RFP Templates

*Three procurement routes, with the questions that surface what a contract will actually say*

EDITION 2026.1 · JULY 2026

| | |
|---|---|
| CONTENTS | Part one, PBM Contract Compliance RFI, tests an incumbent against fiduciary standards. Part two, PBM Market Landscape RFI, maps which PBMs will meet them. Part three, PBM RFP, runs the full procurement |
| HOW TO USE | Take the part that matches where you are. Each is a complete template with its own sections and appendices, written to be adapted rather than sent as-is |
| COMPANION | Pair these with the PBM Disclosure and Attestation letters, and with the model contract language the questionnaires reference |
| ONLINE | nautilushealth.org/pbm · contractxray.com |

## PBM Contract Compliance RFI

### Fiduciary Alignment with ERISA, the CAA, and State Laws

#### SECTION 1: INTRODUCTION AND OBJECTIVES

**1.1 Purpose**

[Company Name] has a fiduciary responsibility to contractually support our obligations under ERISA §404(a) and the Consolidated Appropriations Act (CAA) 2021.

Our goal is to confirm our current PBM agreement meets applicable fiduciary, legal, and transparency standards, reflecting best-in-class governance practices.

This Request for Information (RFI) is not a solicitation for general vendor proposals. It is a targeted review of our current PBM agreement to determine whether it can be brought into full compliance with fiduciary, legal, and transparency standards under ERISA, the CAA, and state laws.

### 1.2 Objectives

The objective of this RFI is to remediate our current pharmacy benefit contract as the basis for a long term partnership. If remediation is not feasible, we may issue a competitive RFP to other vendors able to deliver advances in compliance, clinical outcomes, and financial performance.

Our ongoing vendor relationship must:

- Uphold fiduciary obligations under ERISA and CAA through full compensation disclosure, auditability, and documentation
- Eliminate conflicts of interest via Lowest Net Cost pricing, pass-through arrangements, and P&T committee transparency

- Enable real-time oversight through unrestricted data access, third-party integration, and governance controls
- Align clinical decision-making with participant-first principles: Right Drug → Right Participant → Right Price

This document provides our current vendor an opportunity to affirm alignment with these principles, or to identify specific contract or operational gaps requiring resolution.

## SECTION 2: GUIDING PRINCIPLES

All vendor responses will be evaluated against ten fiduciary-aligned provisions, grouped into three domains:

**Domain 1. Fiduciary Conduct.** Does the PBM act in the plan's interest?

- Fiduciary Loyalty Commitment
- Conflict of Interest and Neutrality
- Lowest Net Cost and Clinical Integrity

**Domain 2. Financial Integrity.** Does the plan's money stay the plan's?

- Pass-Through Pricing Integrity
- Rebate and Manufacturer Revenue
- Administrative Fee Transparency

**Domain 3. Oversight and Control.** Can the plan sponsor see and enforce the deal?

- Data Ownership and Rights
- Audit Rights and Verification
- Carve-Out and Vendor Rights
- Termination and Clean Exit

Each provision is scored from 0 to 100 against published model contract language, and the ten together produce the Fiduciary Alignment Score. Every provision exists so that plan assets are spent solely in participants' best interests.

For more information on these provisions and our fiduciary-aligned procurement framework, see the **Nautilus PBM Field Guide** at [link].

## SECTION 3: SCOPE OF RESPONSE

Respondents must confirm acceptance of or propose modifications to the following elements:

**3.1 Services -** Respondents must confirm their ability to provide the services detailed in Appendix A: Scope of Services.

**3.2 Compliance** – Respondents must confirm their agreement with the questions detailed in Appendix B: Compliance Questionnaire.

**3.3 Contract** – Respondents must confirm their agreement or note requested edits to the model provisions outlined in Appendix 'C' Contract Questionnaire.

These provisions represent our non-negotiable baseline for ongoing engagement.

Upon review of responses and documentation, our fiduciary committee may request additional details, clarification, or documentation as needed.

## SECTION 4: RESPONSE REQUIREMENTS

### 4.1 This RFI uses the Nautilus PBM Configurator to manage vendor responses.

- An Excel response template is attached and must be completed as the official response vehicle for Appendices B and C.
- The responses must be submitted as an Excel file using the naming convention: [companyname_vendorname_MM.DD.YYYY.xls]
- Supplemental materials may be submitted as PDFs.
- Any questions should be directed to: [name, title, email, telephone]
- Your response should be submitted via email to: [name, title, email, telephone]

### 4.2 Timeline

- RFI Release: [Date]
- Questions Due: [Date]
- Response Due: [Date]
- Clarification Discussions: [Date]

Vendor should submit responses by the response due date above. Late submissions may not be considered. The evaluation committee reserves the right to adjust timelines based on responsiveness, regulatory developments, or documentation needs.

## SECTION 5: EVALUATION AND SELECTION

Responses will be evaluated based on their ability to bring the existing contract into alignment with fiduciary and regulatory obligations.

**5.1 Evaluation:** The review will focus on:

- **Contract Integrity & Fiduciary Alignment**
- **Financial Transparency & Competitiveness**
- **Clinical Governance**
- **Operational Evidence & Technology**

## APPENDICES

- **Appendix A: Scope of Services**
- **Appendix B: Compliance Questionnaire**
- **Appendix C: Contract Questionnaire**

We appreciate your partnership in this remediation process. Our goal is not just contract compliance. We are building a relationship rooted in fiduciary trust, clinical integrity, and sustainable value for plan participants.

## APPENDIX A: SCOPE OF SERVICES

Respondents may provide up to 2 pages per service describing their latest capabilities, features, and key performance metrics for each of these required service areas.

- Claims processing and adjudication
- Network access and discounts
- Formulary development and management
- Clinical programs and utilization management
- Data analytics and reporting
- Member and provider services
- Rebate administration
- Other

## APPENDIX B: COMPLIANCE QUESTIONNAIRE

To be considered, Respondents must agree in principle to the Compliance Provisions in the **Nautilus PBM Configurator** file. These terms reflect core fiduciary and transparency standards and will be incorporated into the final agreement, if any.

### Attestation:

By signing below, I affirm the information provided is true, and our organization agrees to uphold these standards.

**Organization:** _____

**Name & Title:** _____

**Signature:** _____

**Date:** _____

### CONTACT INFORMATION:

Please direct any questions about this response to:

**Organization:** _____

**Name & Title:** _____

**Email:** _____

**Telephone:** _____

## APPENDIX C: CONTRACT QUESTIONNAIRE

To be considered, Respondents must agree in principle to the Contract Provisions in the **Nautilus PBM Configurator** file. These terms reflect core fiduciary and transparency standards and will be incorporated into the final agreement, if any.

### Attestation:

By signing below, I affirm the information provided is true, and our organization agrees to uphold these standards.

**Organization:** _____

**Name & Title:** _____

**Signature:** _____

**Date:** _____

## CONTACT INFORMATION:

Please direct any questions about this response to:

**Organization:** _____

**Name & Title:** _____

**Email:** _____

**Telephone:** _____

# PBM Market Landscape RFI

## Mapping the Field of Transparent, Participant-First PBMs

SECTION 1: INTRODUCTION AND OBJECTIVES

### 1.1 Purpose

This Request for Information (RFI) is a solicitation of information for pharmacy benefit management (PBM) services.

### 1.2 Objectives

[Company Name] has a fiduciary responsibility to contractually support our obligations under ERISA §404(a) and the Consolidated Appropriations Act (CAA) 2021. Our goal is to identify and assess PBMs able to deliver advancements in compliance, clinical outcomes, and financial performance.

The ideal PBM must:

- Support our legal obligations under ERISA and the CAA through fiduciary alignment, full compensation disclosure, and auditability
- Eliminate financial conflicts of interest by adopting a Lowest Net Cost methodology, pass-through pricing, and P&T committee transparency
- Enable plan oversight through real-time data access, third-party integration flexibility, and detailed auditability and governance structures
- Prioritize clinical value by aligning formulary and coverage decisions with our guiding principle: Right Drug → Right Participant → Right Price

**We are not seeking status quo administration. We are seeking alignment with fiduciary law, participant interests, and performance-based accountability.**

### 1.3 Company Overview

- Company: [Name, Background)
- HQ Location: [Address, City, State, Zip]
- Plan: [Name, ID]
- Size: [Number employees, covered lives]
- Demographic Profile: [Sex, Age]

### 1.4 Current Pharmacy Program Overview

- Plan Governance Model: [e.g., fiduciary committee, third-party advisor]
- Current PBM: [Name, contract type, carve-in/out]
- Total annual Rx spend: \$[insert amount]
- Total annual Medical spend: \$[insert amount]
- Plan structure: [e.g., tiers, copays, etc.]

- Carve-outs: [e.g., specialty, mail, prior auth, etc.]
- Key Clinical Priorities: [e.g., diabetes, cardiometabolic, mental health]

## SECTION 2: GUIDING PRINCIPLES

All vendor responses will be evaluated against ten fiduciary-aligned provisions, grouped into three domains:

**Domain 1. Fiduciary Conduct.** Does the PBM act in the plan's interest?

- Fiduciary Loyalty Commitment
- Conflict of Interest and Neutrality
- Lowest Net Cost and Clinical Integrity

**Domain 2. Financial Integrity.** Does the plan's money stay the plan's?

- Pass-Through Pricing Integrity
- Rebate and Manufacturer Revenue
- Administrative Fee Transparency

**Domain 3. Oversight and Control.** Can the plan sponsor see and enforce the deal?

- Data Ownership and Rights
- Audit Rights and Verification
- Carve-Out and Vendor Rights
- Termination and Clean Exit

Each provision is scored from 0 to 100 against published model contract language, and the ten together produce the Fiduciary Alignment Score. Every provision exists so that plan assets are spent solely in participants' best interests.

For more information on these provisions and our fiduciary-aligned procurement framework, see the **Nautilus PBM Field Guide** at [link].

## SECTION 3: SCOPE OF RESPONSE

Respondents must confirm acceptance of or propose modifications to the following elements:

**3.1 Services -** Respondents must confirm their ability to provide the services detailed in Appendix A: Scope of Services.

**3.2 Compliance** – Respondents must confirm their agreement with the questions detailed in Appendix B: Compliance Questionnaire.

**3.3 Contract** – Respondents must confirm their agreement or note requested edits to the model provisions outlined in Appendix 'C' Contract Questionnaire.

**3.4 Pricing** – Respondents must provide sample pricing as detailed in Appendix D: Pricing Questionnaire.

**3.5 References** – Respondents must provide two (2) references from clients with similar scope of services as part of Appendix E: Client References

Responses will be reviewed solely for the purpose of compliance and procurement evaluation and will not be shared outside the fiduciary committee or its advisors.

Upon review of vendor responses and documentation, our fiduciary committee may request additional details, clarification, or documentation as needed.

Finalists will be notified in advance of any follow-up interviews or RFP process.

## SECTION 4: RESPONSE REQUIREMENTS

### 4.1 This RFI uses the Nautilus PBM Configurator to manage vendor responses.

- An Excel response template is attached and must be completed as the official response vehicle for Appendices B and C.
- The responses must be submitted as an Excel file using the naming convention: [companyname_vendorname_MM.DD.YYYY.xls]
- Supplemental materials may be submitted as PDFs.
- Any questions should be directed to: [name, title, email, telephone]
- Your response should be submitted via email to: [name, title, email, telephone]

### 4.2 Timeline

- RFI Release: [Date]
- Questions Due: [Date]
- Response Due: [Date]
- Clarification Discussions: [Date]
- Shortlist Announcement: [Date]

Vendors should submit responses by the response due date above. Late submissions may not be considered. Timelines are subject to change based on vendor responsiveness or fiduciary committee review.

## SECTION 5: EVALUATION AND SELECTION

**5.1 Evaluation:** Proposals will be evaluated on the following criteria:

- **Contract Integrity & Fiduciary Alignment – 30%**
- **Financial Transparency & Competitiveness – 20%**
- **Clinical Governance – 20%**
- **Operational Evidence & Technology – 20%**
- **References and Implementation – 10%**

## APPENDICES

- **Appendix A: Scope of Services**
- **Appendix B: Market Questionnaire**
- **Appendix C: Contract Questionnaire**
- **Appendix D: Pricing Questionnaire**
- **Appendix E: Client References**

## APPENDIX A: SCOPE OF SERVICES

Respondents may provide up to 2 pages per service describing their latest capabilities, features, and key performance metrics for each of these required service areas.

- Claims processing and adjudication
- Network access and discounts
- Formulary development and management
- Clinical programs and utilization management
- Data analytics and reporting
- Member and provider services
- Rebate administration
- Other

## APPENDIX B: MARKET QUESTIONNAIRE

To be considered, Respondents must agree in principle to the Market Questions in the **Nautilus PBM Configurator** file. These terms reflect core fiduciary and transparency standards and will be incorporated into the final agreement, if any.

### Attestation:

By signing below, I affirm the information provided is true, and our organization agrees to uphold these standards.

**Organization:** _____

**Name & Title:** _____

**Signature:** _____

**Date:** _____

### CONTACT INFORMATION:

Please direct any questions about this response to:

**Organization:** _____

**Name & Title:** _____

**Email:** _____

**Telephone:** _____

## APPENDIX C: CONTRACT QUESTIONNAIRE

To be considered, Respondents must agree in principle to the Contract Provisions in the **Nautilus PBM Configurator** file. These terms reflect core fiduciary and transparency standards and will be incorporated into the final agreement, if any.

### Attestation:

By signing below, I affirm the information provided is true, and our organization agrees to uphold these standards.

**Organization:** _____

**Name & Title:** _____

**Signature:** _____

**Date:** _____

### CONTACT INFORMATION:

Please direct any questions about this response to:

**Organization:** _____

**Name & Title:** _____

**Email:** _____

**Telephone:** _____

## APPENDIX D: PRICING QUESTIONNAIRE

The pricing appendix is designed to reveal pricing dynamics and eliminate ambiguity around prices, markups, rebates, pass-throughs, etc.

### 1. Three File Transparency Analysis

As a test of transparency, vendors shall submit these four files, at a minimum covering a period of the last 12 months and preferably the last 3 years:

1. **Pharmacy Claims:** NDC, date, quantity, ingredient cost, dispensing fee, plan payment
2. **Rebate and Revenue:** Every dollar received, retained, or passed through
3. **Admin Fee Invoices:** Flat PMPM or per-claim fees, nothing buried in margins

### 2. High Volume and Cost Analysis

The 15 drugs listed below represent significant volume and cost drivers. Please complete all the columns.

| Drug Name | NDC | Scripts / Year | Ingredient Cost | Cost Basis | Admin Fee | Rebates & Other Revenue | Net Cost to Plan |
|---|---|---|---|---|---|---|---|
| Metformin 500mg | 0093-2077-01 | xx,000 | | | | | |
| Atorvastatin 20mg | 0071-0155-23 | xx,000 | | | | | |
| Lisinopril 10mg | 0093-7222-01 | xx,000 | | | | | |

| Drug Name | NDC | Scripts / Year | Ingredient Cost | Cost Basis | Admin Fee | Rebates & Other Revenue | Net Cost to Plan |
|---|---|---|---|---|---|---|---|
| Eliquis 5mg | 0003-0893-21 | x,000 | | | | | |
| Ozempic 1mg/dose | 0169-2837-13 | x,x00 | | | | | |
| Dupixent 300mg | 0024-5581-01 | x,x00 | | | | | |
| Jardiance 25mg | 0597-0275-07 | x,x00 | | | | | |

| Drug Name | NDC | Scripts / Year | Ingredient Cost | Cost Basis | Admin Fee | Rebates & Other Revenue | Net Cost to Plan |
|---|---|---|---|---|---|---|---|
| Humira 40mg/0.8ml | 0074-3799-02 | xx0 | | | | | |
| Skyrizi 150mg | 0074-1092-01 | xx0 | | | | | |
| Entyvio 300mg | 4810-0035-43 | xx0 | | | | | |
| Spinraza 12mg | 0078-0800-61 | x0 | | | | | |

| | | |
|---|---|---|
| Insulin Lantus 100u/ml | 0088-2220-33 | x,x00 |
| Advair Diskus 250/50mcg | 0173-0695-20 | x,x00 |
| EpiPen 0.3mg | 0049-0220-02 | x,x00 |

### 3. Category-Specific Methodology

For each drug category above, explain:

**Generic Drugs:**

- MAC pricing determination process
- Guaranteed cost basis (NADAC, other)
- Frequency of pricing updates

**Brand Drugs:**

- Rebate calculation and pass-through timeline
- Additional manufacturer revenue identification

- Net cost determination process

Specialty Drugs:

- Specialty classification criteria (must be FDA-based)
- All revenue sources and fee structures
- Patient assistance program coordination

### 4. Revenue Transparency Summary

Complete for all revenue sources across your proposed arrangement:

| |
|---|
| Drug Ingredient Markup |
| Dispensing Fees Retained |
| Manufacturer Rebates |
| Manufacturer Administrative Fees |
| Network Pharmacy Revenue Sharing |
| Data Sales/Analytics Revenue |
| Specialty Pharmacy Margin |
| Other (Specify) |

### 5. Administrative Fees (Itemized)

| |
|---|
| Claims Adjudication |
| Clinical Program Management |
| Reporting Tools |
| Customer Service |
| Audit/Data Access Support |
| Other (Specify) |

### Instructions:

- Use actual pricing examples where possible
- Hypothetical or non-market-based pricing will be deemed non-responsive
- All NDC-level pricing must reflect current marketplace arrangements
- Provide specific cost basis for each drug category
- Disclose ALL revenue sources, including indirect remuneration

## APPENDIX E: CLIENT REFERENCES

Respondents must provide at least 2 client references of companies with similar size and scope of services.

# PBM RFP

## Building a Fiduciary-Aligned and Transparent PBM Partnership

SECTION 1: INTRODUCTION AND OBJECTIVES

### 1.1 Purpose

This RFP is a solicitation for vendor proposals for pharmacy benefit management (PBM) services.

### 1.2 Objectives

[Company Name] has a fiduciary responsibility to contractually support our obligations under ERISA §404(a) and the Consolidated Appropriations Act (CAA) 2021. Our goal is to select a PBM partner who can measurably advance legal compliance, participant outcomes, clinical stewardship, and financial performance.

Our selected PBM must:

- Support our legal obligations under ERISA and the CAA through fiduciary alignment, full compensation disclosure, and auditability
- Eliminate financial conflicts of interest by adopting a Lowest Net Cost methodology, pass-through pricing, and P&T committee transparency
- Enable plan oversight through real-time data access, third-party integration flexibility, and detailed auditability and governance structures
- Prioritize clinical value by aligning formulary and coverage decisions with our guiding principle: Right Drug → Right Participant → Right Price

**We are not seeking incremental improvements or status quo administration. We are seeking a fiduciary-aligned PBM partner committed to full transparency, participant-first governance, and measurable performance accountability.**

### 1.3 Company Overview

- Company: [Name, Background)
- HQ Location: [Address, City, State, Zip]
- Plan: [Name, ID]
- Size: [Number employees, covered lives]
- Demographic Profile: [Sex, Age]

### 1.4 Current Pharmacy Program Overview

- Plan Governance Model: [e.g., fiduciary committee, third-party advisor]
- Current PBM: [Name, contract type, carve-in/out]
- Total annual Rx spend: \$[insert amount]
- Total annual Medical spend: \$[insert amount]

- Plan structure: [e.g., tiers, copays, etc.]
- Carve-outs: [e.g., specialty, mail, prior auth, etc.]
- Key Clinical Priorities: [e.g., diabetes, cardiometabolic, mental health]

### 1.5 Current Pharmacy Performance Baseline

All proposals must clearly reference how the proposed solution will improve these baseline measures within the first contract year.

**Financial Metrics**:

- Total annual pharmacy spend: \$[X]
- Average cost per script: \$[X] (generic: \$[X], brand: \$[X], specialty: \$[X])
- Current rebate performance: \$[X] annually ([X]% of brand spend)
- Administrative fees: \$[X] annually
- Member out-of-pocket costs: \$[X] annually

**Utilization Metrics**:

- Generic utilization rate: [X]%
- Specialty drug percentage: [X]% of scripts, [X]% of cost
- Mail-order utilization: [X]% - 90-day prescription utilization: [X]%

**Clinical Performance**:

- Adherence rates by therapeutic class (see Appendix D)
- Clinical program participation rates
- Prior authorization approval rates: [X]%
- Generic substitution rates: [X]%

**Network and Access**:

- Network pharmacy utilization patterns
- Geographic access metrics
- Member satisfaction scores: [X]/10
- Pharmacy complaints: [X] per 1,000 members

**Performance Improvement Targets**:

- Cost reduction goals: [X]% net cost improvement
- Clinical outcome improvements
- Member satisfaction enhancement
- Administrative efficiency gains

## SECTION 2: GUIDING PRINCIPLES

All vendor responses will be evaluated against ten fiduciary-aligned provisions, grouped into three domains:

**Domain 1. Fiduciary Conduct.** Does the PBM act in the plan's interest?

- Fiduciary Loyalty Commitment
- Conflict of Interest and Neutrality
- Lowest Net Cost and Clinical Integrity

**Domain 2. Financial Integrity.** Does the plan's money stay the plan's?

- Pass-Through Pricing Integrity
- Rebate and Manufacturer Revenue
- Administrative Fee Transparency

**Domain 3. Oversight and Control.** Can the plan sponsor see and enforce the deal?

- Data Ownership and Rights
- Audit Rights and Verification
- Carve-Out and Vendor Rights
- Termination and Clean Exit

Each provision is scored from 0 to 100 against published model contract language, and the ten together produce the Fiduciary Alignment Score. Every provision exists so that plan assets are spent solely in participants' best interests.

For more information on these provisions and our fiduciary-aligned procurement framework, see the **Nautilus PBM Field Guide** at [link].

## SECTION 3: SCOPE OF RESPONSE

Respondents must confirm acceptance of or propose modifications to the following elements:

**3.1 Services -** Respondents must confirm their ability to provide the services detailed in Appendix A: Scope of Services.

**3.2 Compliance** – Respondents must confirm their agreement with the questions detailed in Appendix B: Compliance Questionnaire.

**3.3 Contract** – Respondents must confirm their agreement or clearly identify and explain any requested deviations from the model contract provisions outlined in Appendix 'C' Contract Questionnaire.

**3.4 Pricing** – Respondents must provide sample pricing as detailed in Appendix D: Pricing Questionnaire.

**3.5 References** – Respondents must provide two (2) references from clients with similar scope of services as part of Appendix E: Client References

Upon review of vendor responses and documentation, our fiduciary committee may request additional details, clarification, or documentation as needed.

Finalists will be notified in advance of any follow-up interviews.

## SECTION 4: RESPONSE REQUIREMENTS

### 4.1 This RFI uses the Nautilus PBM Configurator to manage vendor responses.

- An Excel response template is attached and must be completed as the official response vehicle for Appendices B and C.

- The responses must be submitted as an Excel file using the naming convention: [companyname_vendorname_MM.DD.YYYY.xls]
- Supplemental materials may be submitted as PDFs.
- Any questions should be directed to: [name, title, email, telephone]
- Your response should be submitted via email to: [name, title, email, telephone]

### 4.2 Submission Process and Contact

Your response should be submitted via email to:

[Contact Name]
[Title]
[Email Address]
[Phone Number]

### 4.3 Timeline

- RFP Release: [Date]
- Questions Due: [Date]
- Proposals Due: [Date]
- Finalist Presentations: [Insert Date]
- Clarification Discussions: [Date]
- Anticipated Decision: [Date]
- Implementation Start Date: [Date]

Vendors should submit responses by the Proposal Due Date above. Late submissions may not be considered. Timelines are subject to change based on vendor responsiveness or fiduciary committee review.

## SECTION 5: PROPOSAL INSTRUCTIONS

5.1 Required Responses

Proposals must include the following (maximum 50 pages excluding appendices):

1. **Technical Proposal (20 pages)**:

- Executive Summary (2 pages): Fiduciary alignment and unique value proposition
- Service Delivery Approach (4 pages): Detailed methodology for each service area
- Clinical and Formulary (4 pages): Decision making and LNC methodology
- Technology (4 pages): API capabilities, integration examples, real-time reporting
- Implementation (3 pages): Timeline, risk mitigation, and transition strategy
- Performance (3 pages): Monitoring, guarantees, and continuous improvement

1. **Financial Proposal (15 pages)**:

- Comprehensive Pricing (8 pages): Complete cost structure
- Cost-Benefit Analysis (4 pages): Projected savings and value including:

- Baseline comparison using current performance data
- Year 1, 2, and 3 projected performance improvements
- ROI calculation methodology and assumptions
- Risk-adjusted savings projections
- Sensitivity analysis for key variables (e.g., GLP-1 increases, etc.)
- Implementation costs and timeline to break-even
- Guarantees (3 pages): Performance commitments and penalty structure

1. **Supporting Documentation (5 pages)**:

- Contract Terms Acceptance: Response to all Appendix C requirements
- Reference Summary: Detailed case studies from similar implementations
- Organizational Capability: Team qualifications and capacity confirmation

### 5.2 Continuity with RFI Process

If you participated in the prior RFI process, your proposal must:

- Address any gaps, clarifications, or concerns identified during the RFI
- Include the most current version of your PBM Configurator response
- Clearly indicate any updated information since the RFI submission

## SECTION 6: EVALUATION AND SELECTION

**6.1 Evaluation:** Proposals will be evaluated on the following criteria:

- **Contract Integrity & Fiduciary Alignment – 30%**
- **Financial Transparency & Competitiveness – 20%**
- **Clinical Governance – 20%**
- **Operational Evidence & Technology – 20%**
- **References and Implementation – 10%**

Respondents should carefully review and complete all of the following appendices, as they form the contractual foundation for evaluation and selection. Incomplete responses may be excluded from consideration.

Any proposal that fails to affirmatively accept the fiduciary standards and contract provisions outlined in Appendix C and the PBM Configurator may be disqualified from further consideration.

## APPENDICES

- **Appendix A: Scope of Services**
- **Appendix B: Compliance Questionnaire**
- **Appendix C: Contract Questionnaire**
- **Appendix D: Pricing Questionnaire**
- **Appendix E: Client References**

- **Appendix F: – Implementation Requirements and Timeline Templates**
- **Appendix G: – Performance Standards and Guarantee Structure**
- **Appendix H: – Technology Integration Requirements and API Specifications**

## APPENDIX A: SCOPE OF SERVICES

1. **Core Services**

Respondents may provide up to 2 pages per service describing their latest capabilities, features, and key performance metrics for each of these required service areas.

- Claims processing and adjudication
- Network access and discounts
- Formulary development and management
- Clinical programs and utilization management
- Data analytics and reporting
- Member and provider services
- Rebate administration
- Other

1. **Value-Added Services**

Respondents may provide up to 2 pages per service describing their latest capabilities, features, and key performance metrics for each of these optional service areas.

- Specialty drug management and optimization
- Pharmacy-based chronic condition management
- Biosimilar adoption and education
- Clinical Effectiveness Review
- GLP-1 Management
- Copay optimization and affordability
- Stop-loss and point solution integration
- Support for fiduciary and board-level reporting

## APPENDIX B: COMPLIANCE QUESTIONNAIRE

To be considered, Respondents must agree in principle to the Compliance Provisions in the **Nautilus PBM Configurator** file. These terms reflect core fiduciary and transparency standards and will be incorporated into the final agreement, if any.

### Attestation:

By signing below, I affirm the information provided is true, and our organization agrees to uphold these standards.

**Organization:** _____

**Name & Title:** _____

**Signature:** _____

**Date:** _____

### CONTACT INFORMATION:

Please direct any questions about this response to:

**Organization:** _____

**Name & Title:** _____

**Email:** _____

**Telephone:** _____

## APPENDIX C: CONTRACT QUESTIONNAIRE

To be considered, Respondents must agree in principle to the Contract Provisions in the **Nautilus PBM Configurator** file. These terms reflect core fiduciary and transparency standards and will be incorporated into the final agreement, if any.

### Attestation:

By signing below, I affirm the information provided is true, and our organization agrees to uphold these standards.

**Organization:** _____

**Name & Title:** _____

**Signature:** _____

**Date:** _____

### CONTACT INFORMATION:

Please direct any questions about this response to:

**Organization:** _____

**Name & Title:** _____

**Email:** _____

**Telephone:** _____

## APPENDIX D: PRICING QUESTIONNAIRE

All RFI drugs (15) plus additional 35 drugs from current top 50

1. Detailed cost modeling for projected 3-year performance
2. Scenario analysis (volume changes, new drug launches, formulary changes)
3. Administrative fee breakdown by service component
4. Technology and implementation cost schedules
5. Performance guarantee cost impact analysis

## APPENDIX E: CLIENT REFERENCES

Respondents must provide at least 2 client references of companies with similar size and scope of services.

## APPENDIX F: IMPLEMENTATION AND TRANSITION REQUIREMENTS

Proposals must address:

- Implementation timeline (90 days with key milestones)
- Technical integration approach and testing protocols
- Transition management from current PBM
- Risk mitigation and contingency planning
- Post-implementation support structure

## APPENDIX G: PERFORMANCE STANDARDS AND GUARANTEES

Proposals must commit to the following measurable standards with monetary penalties for non-compliance:

### Clinical Performance:

- Generic substitution rate: [X]% minimum
- Prior authorization response: 95% within 24 hours
- Clinical program enrollment targets
- -Adherence improvement metrics

**Network Access**:

- Network pharmacy access: 95% within 15 miles (urban), 30 miles (rural)
- Specialty pharmacy access standards
- Mail-order delivery performance: 95% within 3 business days

**Claims Processing**:

- Claims processing accuracy: 99.5% minimum
- Claims adjudication speed: Real-time for 98% of claims
- Claims payment timeliness: 98% within 48 hours

**Data and Transparency**:

- Data access response time: <24 hours for standard requests
- Audit accommodation rate: 100% within agreed timelines
- Financial reconciliation completion: Within 30 days of period end
- API response time: <2 seconds for standard queries

### Technology Platform Requirements:

- API response times: <2 seconds for standard queries, <5 seconds for complex reports
- Data feed capabilities: Real-time claims, daily financial reconciliation, weekly clinical reports
- Integration protocols: RESTful APIs, secure FTP, HL7 FHIR compatibility

- Third-party vendor connectivity: Must support [specific vendors from RFI]
- Security standards: SOC 2 Type II, HIPAA compliance, encryption protocols
- Uptime guarantee: 99.9% availability with penalty structure

**Customer Service**:

- Member call answer rate: 90% within 60 seconds
- Member issue resolution: 95% within 24 hours
- Provider call answer rate: 85% within 90 seconds
- Complaint resolution: 98% within 5 business days

**Penalty Structure**:

- Claims processing failures: \$500 per percentage point below target per month
- Data access delays: \$100 per day per delayed request
- Customer service failures: \$200 per percentage point below target per month
- Clinical performance shortfalls: \$1,000 per percentage point below target per quarter
- Escalating penalties: 50% increase for consecutive monthly failures
- Maximum penalties: Cannot exceed 25% of monthly fees
- Termination threshold: Three consecutive months of multiple failures

## APPENDIX H: TECHNOLOGY SPECIFICATIONS

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